HART Hub Triage Connection Tool

Coordinator-Facing | Healing & Treatment

Quick rule: Based on the needs assessment, make **at least one referral or connection** if the person consents. The receiving service confirms fit.

STEP 1 — IMMEDIATE SAFETY SCREEN (NOW)

If YES to any, do same-day escalation first, then return to triage plan.

- Immediate medical concern / needs ER
- Acute intoxication / withdrawal risk needing medical review
- Current risk of harm to self / others (or cannot care for self right now)
- Severe confusion / disorientation

Action:

- Call appropriate crisis / medical response
- Arrange transport support if needed (taxi / ambulance / other)
- Document Incident Report + follow-up plan
- Safe to transport / Need assistance

STEP 2 — CONSENT CHECKPOINT

- HART Hub written consent completed
- Consent covers service coordination + referral sharing among HART Hub partners
- Client preferences recorded (contact method, cultural supports, goals)

STEP 3 — TRIAGE DOMAINS (CHECK + CONNECT)

For each domain: Check need → pick at least one connection.

1) BASIC NEEDS TODAY

Need Signals	Connections
Food / clothing / hygiene / laundry	Onsite support / navigation
Transportation help	Transportation coordinator (if applicable)
ID needs	

2) HOUSING INSTABILITY

Need Signals	Connections
Unsheltered / couch surfing	Housing navigation (onsite)
Using overnight shelter	Supportive Housing pathway (Norwood / WNHAC, if aligned)
Eviction risk	Rent supplement pathway (if aligned)
	WIN Shelter coordination / transfer (if aligned)

3) MENTAL HEALTH SUPPORT

Need Signals	Connections
Distress impacting daily function	Crisis stabilization worker (if urgent but not ED)
Recurrent crisis calls / repeated ED use (if known)	Partner mental health pathway (external referral as needed)
	Intensive Case Management or Adult Case Management

4) SUBSTANCE USE SUPPORT

Need Signals	Connections
■ Wants help / change	■ Addictions specialist (onsite)
■ Harm concerns / overdose history (if reported)	■ Coordinate referral to RAAM (standalone partner pathway)

5) ONGOING COORDINATION NEEDED (COMPLEXITY)

Need Signals	Connections
■ Multiple systems involved / 'no home service'	■ HART Hub case management / transition support
■ Needs frequent follow-up	■ KACL Intensive Case Management referral (KACL confirms fit)

6) CULTURAL AND COMMUNITY CONNECTION (TWO-EYED SEEING)

Need Signals	Connections
■ Client wants Indigenous-led supports	■ Plan reflects client goals
■ Client wants Western clinical supports	
■ Client wants both	

STEP 4 — SERVICE TIER (INTERNAL PLANNING)

Use as staffing guidance (not eligibility).

■ **Tier 1:** Peer support + navigation + transport + cultural + hub coordinator + primary care
■ **Tier 2:** Add case management + addiction specialist
■ **Tier 3:** Add crisis stabilization + ICM

STEP 5 — REFERRAL ACTIONS

Complete one row per referral.

Service	Method	Consent Shared	Follow-up Date	Outcome
_____	■ Warm handoff ■ Phone ■ Email ■ Fax ■ CareTeam	■ Yes ■ No	_____	■ Accepted ■ Pending ■ Declined ■ Redirected
_____	■ Warm handoff ■ Phone ■ Email ■ Fax ■ CareTeam	■ Yes ■ No	_____	■ Accepted ■ Pending ■ Declined ■ Redirected
_____	■ Warm handoff ■ Phone ■ Email ■ Fax ■ CareTeam	■ Yes ■ No	_____	■ Accepted ■ Pending ■ Declined ■ Redirected
_____	■ Warm handoff ■ Phone ■ Email ■ Fax ■ CareTeam	■ Yes ■ No	_____	■ Accepted ■ Pending ■ Declined ■ Redirected
_____	■ Warm handoff ■ Phone ■ Email ■ Fax ■ CareTeam	■ Yes ■ No	_____	■ Accepted ■ Pending ■ Declined ■ Redirected

REFERRAL ORDERING — DECISION TREE AND PRIORITY RULES

Decision Tree

1. What is the presenting issue?
2. Does internal service cover this? Yes → Refer internally. No → next branch.
3. Is there a local partner who accepts this referral? Yes → Refer locally.
4. If no → escalate to specialized referral list.

Referral Matrix

Primary Need	First Referral	Second Referral	Third Referral	Notes
Dual-Diagnoses / Complex Needs	Intensive Case Management	Addiction Specialist	LWDH Interagency Form	Urgent cases bypass to crisis
Transportation	Transportation Coordinator	Community detox	Live-In Treatment	
Cultural Supports	KCA Adult MH	Neechee	Other Local or Community Partners	

Priority Rules

Rule 1: Always use internal services first when appropriate.
Rule 2: Safety concerns override the hierarchy (go straight to crisis services).
Rule 3: If multiple options fit, choose the least intrusive and most culturally responsive option first.
Rule 4: For medical issues, defer to primary care immediately.
Rule 5: If the client requests a specific culturally responsive provider, prioritize that unless unsafe.